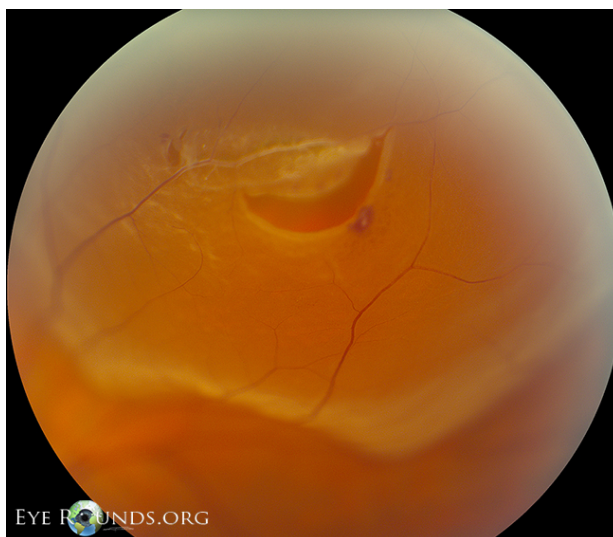


Three mechanisms separate the neurosensory retina

A detached retina is separated from the RPE; the cause determines how that separation occurs.



A horseshoe tear with surrounding detachment illustrates a rhegmatogenous mechanism.

Clinical meaning

Rhegmatogenous detachment admits liquefied vitreous through a retinal break. Tractional detachment follows contraction of membranes, often in proliferative diabetes. Exudative detachment accumulates leaking subretinal fluid without a break or traction. High myopia, flashes, floaters and a peripheral curtain support a break-related presentation.

A break and an established detachment have different needs

Macular involvement changes visual risk; an acutely symptomatic detachment needs urgent referral.

Break or detachment?

Inspect the retina around the lesion

Break with attached retina

Retinopexy forms a chorioretinal adhesion around the break.

Established detachment

Reattachment management may use vitrectomy or scleral buckling.

Clinical meaning

An acutely symptomatic detachment needs urgent referral, particularly while the macula remains attached. Direct fundoscopy can miss a peripheral lesion. Retinopexy relies on contact with underlying tissue and is not a substitute for established-detachment repair. Explain new flashes, floaters, curtain or central blur as urgent return symptoms.